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Director [PENDING CONFIRMATION]
Office of Science & Technology Policy
The White House
1600 Pennsylvania Ave NW
Washington, DC 20500

Sethuraman Panchanathan
Director
National Science Foundation
2415 Eisenhower Avenue
Alexandria, VA 22314

Attention: Faisal D'Souza, NCO and NITRD Program

Subject: **RFI - The Development of an Artificial Intelligence (AI) Action Plan**
[Federal Register 2025-02305. <https://federalregister.gov/d/2025-02305>]

Submitted via email at [REDACTED]

Dear Directors Kratsios and Panchanathan,

On behalf of Whiterabbit.ai (Whiterabbit or the company), thank you for the opportunity to respond to your recent Request for Information (RFI) on the Development of an Artificial Intelligence (AI) Action Plan. We appreciate your efforts to strengthen U.S. leadership in AI and secure a brighter future for all Americans. We fully support your goals to advance human well-being, economic growth, and national security through this AI Action Plan.

Founded in 2017, Whiterabbit.ai is an innovative, venture-backed AI startup based in Silicon Valley. Our leadership team comprises experienced healthcare executives, leading scientists and successful technology entrepreneurs. Our board includes industry leaders across health, software, insurance, outpatient radiology, and breast cancer research. As AI innovators, we are developing AI medical devices that require U.S. Food & Drug Administration (FDA) oversight and market authorization.

We offer the following input to the development of an AI Action Plan, as directed by Executive Order 14179 (Removing Barriers to American Leadership in Artificial Intelligence) with a specific focus on AI in healthcare.



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I. AI Innovation in Healthcare Leads to Wider AI Dominance

AI innovation in healthcare is pivotal to establishing the United States as a global leader in artificial intelligence, driving economic growth, technological advancement, and strategic dominance. Healthcare is a high-impact sector where AI applications, such as predictive analytics, automated diagnostics, and personalized medicine, showcase the transformative potential of machine learning and deep learning models.¹ As the U.S. pioneers AI-driven advancements in medical imaging, drug discovery, robotic-assisted surgery, and virtual health monitoring, it strengthens its position as the epicenter of AI research and commercialization.²

Integrating AI into healthcare not only enhances patient outcomes and operational efficiency but also attracts substantial investment from both the private and public sectors, fueling further innovation.³ With healthcare spending in the United States continuing to rise, leveraging AI to augment healthcare providers presents a critical solution that does not require a compromise on healthcare quality or access to increase cost efficiency. The growing physician shortage, projected by the Association of American Medical Colleges to grow to 86,000 physicians by 2036,⁴ underscores the urgent need for this administration to support AI solutions that bolster the capabilities of American physicians.

Moreover, leadership in AI-powered healthcare gives the U.S. a competitive edge in the global AI landscape. It enables the U.S. to set industry standards, shape regulatory frameworks, and export cutting-edge medical technologies.⁵ Countries that command AI-driven healthcare will not only have stronger healthcare systems but will also control critical AI infrastructure, intellectual property, and data ecosystems that underpin broader AI applications.⁶

II. Game-Changing Healthcare Innovation is Stifled by Antiquated Regulations and Outdated Interpretations Thereof

AI has demonstrated its potential to significantly reduce healthcare costs while improving healthcare outcomes.⁷ Patients are now more willing and enthusiastic than ever for AI to perform diagnostic tasks, and physicians have been incorporating AI into their workflows since the late 1990s.⁸

¹ Topol, E. (2019). *Deep Medicine: How Artificial Intelligence Can Make Healthcare Human Again*. Basic Books

² National Academy of Medicine. (2021). *Artificial Intelligence in Healthcare: The Hope, The Hype, The Promise, The Peril*. Washington, DC.

³ McKinsey & Company. (2023). *The Future of AI in Healthcare: Transforming Patient Care and Industry Growth*.

⁴ Association of American Medical Colleges. (2024). *New AAMC Report Shows Continuing Projected Physician Shortage*.

⁵ Executive Office of the President. (2020). *American Artificial Intelligence Initiative: Year One Annual Report*.

⁶ World Economic Forum. (2022). *AI-Driven Healthcare: Opportunities and Risks in a Global Context*.

⁷ Abramoff et al. (2022). *A reimbursement framework for artificial intelligence in healthcare*. Npj Digital Medicine.

⁸ Summary of Safety and Effectiveness Data: R2 Technologies (P970058) 1998. P970058. U. S. Food and Drug Administration.



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However, there are existing regulations originally intended to prevent unqualified persons from performing medical tasks that may inadvertently restrict the use of FDA-authorized AI technologies. When interpreted literally, these regulations could preclude qualified AI from performing specific healthcare tasks, despite FDA approval and clear clinical benefits. Regulatory ambiguity creates confusion about whether AI can be used for a certain task which discourages companies from pursuing the most innovative technologies and deters investors from supporting them.

A salient example of this challenge is the Mammography Quality Standards Act (MQSA). Enacted in 1992, MQSA aimed to ensure that all women have access to high-quality mammography services by establishing uniform standards for mammography facilities across the United States. The Act addressed concerns such as poor quality equipment, inadequately trained staff, and inconsistent quality assurance procedures. Its ultimate goal was to improve early breast cancer detection through standardized, quality imaging practices.⁹ Among these standards and requirements were the following:

- 42 USC § 263b(f)(1)(D): ... *a requirement that **mammograms be interpreted by a physician** who is certified as qualified to interpret radiological procedures, including mammography...*
- 42 USC § 263b(f)(1)(G)(ii)(I): ... *a requirement that a facility must assure the preparation of a written report of the results of any mammography **examination signed by the interpreting physician**;*

At the time of MQSA's introduction, there was a concern about nurse practitioners or other inexperienced clinicians reading mammograms. AI-based technologies (e.g. deep learning) were still in their infancy and as a result, the law did not anticipate advancements such as expert-level computer algorithms. This includes AI systems that could be evaluated by the FDA and shown to improve radiologist accuracy while significantly reducing their workload through autonomous screening assessment.

However, the language within MQSA could be interpreted to prohibit the clinical use of such AI devices, even if they meet the FDA's safety and efficacy requirements.

Another example of potential regulatory impediment is Section 503(b) of Federal Food, Drug, and Cosmetic Act (21 U.S.C. 353(b)), which requires that a drug be prescribed only by a "practitioner licensed by law to administer such drug." This regulation could prevent the use of an AI for prescribing a drug even if the AI demonstrated exceptional safety and effectiveness and was authorized by the FDA for that purpose.

⁹ CDC, MMWR, Weekly, August 19, 1994 / 43(32);596-597 [Quality Standards: Mammography Facilities](#)



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Similarly, in the Social Security Act, 42 CFR § 410.23(b) “Condition for coverage of screening for glaucoma” states:

- 42 CFR § 410.23(b): *Medicare Part B pays for glaucoma screening examinations provided to eligible beneficiaries as described in paragraph (a)(2) of this section if they are furnished by or under the direct supervision in the office setting of an optometrist or ophthalmologist who is legally authorized to perform these services under State law (or the State regulatory mechanism provided by State law) of the State in which the services are furnished, as would otherwise be covered if furnished by a physician or incident to a physician's professional service.*

In this case, the development and adoption of new AI-based technology could make screening services for glaucoma far more accessible at a more affordable cost. However, such a future may be stifled if equivalent services provided by advanced AI are implicitly excluded from coverage due to legacy statutory language.

III. To Enhance AI Innovation in the Healthcare Sector, We Recommend a Broad Review of Regulations Implicitly Excluding AI

To enhance AI innovation in the healthcare sector, we recommend the following AI Action Plan.

The plan should initiate a broad review of regulations that may *implicitly* exclude AI in healthcare, with priority given to those with greater population impact. While no comprehensive federal regulations explicitly exclude the use of AI in healthcare, many may inadvertently exclude autonomous AI, such as in the case with MQSA. This review will help prioritize necessary regulatory adjustments or reinterpretations to maximize AI’s positive impact on health and prosperity.

To remain a global leader in AI and healthcare innovation, the United States needs a clear directive from this administration that disentangles American AI innovators from legacy laws. If the FDA determines that a device is safe and effective in performing a function independently of a physician, regulations stating or implying that “only a trained physician can perform this function” should be interpreted to include artificial intelligence and machine learning technologies. America’s most innovative healthcare companies developing cutting-edge, AI medical devices are facing existential risk because their innovations have advanced beyond what legislators could imagine when regulations were written decades ago.

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Thank you for the opportunity to comment on the development of an AI Action Plan . Please contact me directly at [REDACTED] or [REDACTED] if you have any questions on our comments.

Sincerely,

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Vice President, Regulatory Affairs and Quality
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